BASELINE → TREATMENT → FOLLOW-UP

# Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

| Check | Before starting | Each treatment cycle | Between doses | Decision / response review | Completion / change |
|---|---|---|---|---|---|
| **Blood count / kidneys / liver** | Baseline bloods; DPD before fluoropyrimidine | Repeat before each cycle | Earlier tests if unwell, dehydrated or toxicity develops | Results and recovery guide any delay or dose change | Follow unresolved abnormalities |
| **Symptoms and function** | Document existing symptoms and usual activity | Review worst symptoms since the last treatment | Use urgent contacts; do not wait for the next visit | Balance benefit, side effects and priorities | Agree recovery care and ongoing support |
| **Blood pressure / urine / wounds** | BP, urine protein and wound/ procedure review | BP before and after treatment; urine protein | Report bleeding, severe headache or abdominal pain urgently | Additional urine tests and specialist review if abnormal | Confirm wound-healing and procedure plan |
| **Heart checks** | Review cardiac history; ECG when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen. | Ask about new chest pain or breathlessness | Chest pain needs urgent assessment | Repeat heart tests if clinically indicated | Follow any heart problem; no fixed echo series |
| **Response / scans** | Confirm cancer setting, disease assessment and intended duration | Review clinical benefit and symptoms | New symptoms may bring review forward | Consultant-directed scans and response assessment | Agree follow-up, completion or another treatment |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00449 v6a · official regimen

THE WHOLE TREATMENT JOURNEY

# FOLFIRI + bevacizumab: treatment and checks

## Metastatic colorectal cancer

Day 1: bevacizumab, irinotecan, folinic acid and fluorouracil, then a 46-hour fluorouracil pump. A cycle lasts 14 days; your team confirms the next treatment.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Each treatment cycle | Between doses | Decision / response review | Completion / change |
|---|---|---|---|---|---|
| **Treatment plan** NCCP 00449 | Confirm indication, medicines and duration | Give the agreed cycle only after review | Follow pump / tablet and symptom instructions | Continue, modify or stop after consultant review | Agree recovery and follow-up |
| **Blood count / kidneys / liver** NCCP 00449 | Baseline bloods; DPD before fluoropyrimidine | Repeat before each cycle | Earlier tests if unwell, dehydrated or toxicity develops | Results and recovery guide any delay or dose change | Follow unresolved abnormalities |
| **Symptoms and function** NCCP 00449 | Document existing symptoms and usual activity | Review worst symptoms since the last treatment | Use urgent contacts; do not wait for the next visit | Balance benefit, side effects and priorities | Agree recovery care and ongoing support |
| **Blood pressure / urine / wounds** NCCP 00449 | BP, urine protein and wound/ procedure review | BP before and after treatment; urine protein | Report bleeding, severe headache or abdominal pain urgently | Additional urine tests and specialist review if abnormal | Confirm wound-healing and procedure plan |
| **Heart checks** NCCP 00449 | Review cardiac history; ECG when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen. | Ask about new chest pain or breathlessness | Chest pain needs urgent assessment | Repeat heart tests if clinically indicated | Follow any heart problem; no fixed echo series |
| **Response / scans** NCCP 00449 | Confirm cancer setting, disease assessment and intended duration | Review clinical benefit and symptoms | New symptoms may bring review forward | Consultant-directed scans and response assessment | Agree follow-up, completion or another treatment |

# What each check is for

### 1 Confirm the treatment aim

Before starting

FOLFIRI + bevacizumab is used for cancer of the colon or rectum that has spread. Treatment aims to control the cancer; the likely benefit and review plan depend on your individual situation. Surgery may be reconsidered in selected cases after multidisciplinary review.

**Ask / act:** Confirm the exact indication and intended duration.

Sources: NCCP 00449

### 2 Review before continuing

At each clinical decision point

Results, symptoms and benefit can change the plan.

**Ask / act:** Agree who reviews results and communicates the next step.

Sources: NCCP 00449

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.21